◆ MEDTECH INNOVATION MAP · STUDENT EDITION

After the Hand-In

You just closed out a course project or a thesis with real health-tech potential. This is the same DTU Skylab map used to connect students with the university hospital — but read as a set of doors, keyed to how far your work already got. Find your entry point below, then follow the track from there.

IF YOU HAVE

An idea, no build yet

Your report identified a real problem — a workflow gap, a device limitation — but nothing physical or coded exists.

→ enter at 02 · Explore & Learn

IF YOU HAVE

A working prototype

Your thesis produced something that actually ran — once, in a lab, in front of an exam committee.

→ enter at 03 · Build & Validate

IF YOU HAVE

A near-market thesis

Your supervisor or examiner said some version of "this could actually be a product."

→ enter at 05 · Launch

CLINICAL COLLABORATOR · DTU SKYLAB · JOINT TOUCHPOINT · FUNDING

adoption raises the next clinical question ↺ back to spark

START: IDEA · START: PROTOTYPE · START: THESIS

01 SPARK	02 EXPLORE & LEARN	03 BUILD & VALIDATE	04 FUND	05 LAUNCH	06 IMPACT
ALREADY BEHIND YOU **Noticing the gap** Your course or thesis already did this part — a clinical need or curiosity is what started the project.	CLINICAL COLLABORATOR **Find a sounding board** Ask a supervisor or Skylab coach to connect you with a clinician who'll pressure-test the idea against real patient-safety constraints.	CLINICAL COLLABORATOR **Test it where it'll live** Get the prototype in front of clinicians for feedback against real workflows — not just an exam committee.	CLINICAL COLLABORATOR **Ask about in-kind support** Equipment access, anonymised data, or staff time — if a department wants your solution to exist, this is what they can offer.	JOINT **Clinical pilot** Get a hospital department to trial the near-market solution under real operating conditions.	OUTCOME **The thesis becomes a spin-out or an adopted tool** Your degree ends; the product doesn't — and it raises the next clinical question for whoever picks it up after you.
DTU SKYLAB **In-Flight & Info Sessions** Skip these if your project already gave you the idea — they're for starting from zero.	JOINT · KEY BRIDGE **Life Science Academy** Bring your report, leave with a collaborator: it matches health-focused student ideas with researchers and clinicians.	JOINT **Skylab Minisprint** Pitch a hospital department on sponsoring a sprint to harden the prototype with real industry partners.	FUNDING **Skylab Funding** Soft funding for prototyping, testing & consultancy — open to students, researchers, **and alumni**. `DKK 15,000–150,000`	DTU SKYLAB **Prototype2Market** Developer Hall residency, lab access, and a technical roadmap — built for research-based startups, which is exactly what a strong thesis produces.	
	DTU SKYLAB **Show up with your thesis** Open Door Coaching — no application, just bring the PDF · EXPLORE Programme `2 mo · biannual` · Pre-startup Mentoring · Advisory Days.	DTU SKYLAB **Keep building, now without a deadline** Tech Support + Developer Hall for continued engineering help, feeding into the 3-stage **Skylab Startup Programme** if you want to turn this into a company.	FUNDING **Research-to-Business Grants** Discovery Grants · Proof of Concept Grants · InnoExplorer — built for exactly this: turning thesis research into a venture.		

WHAT TO BRING TO THE FIRST MEETING

- Your report or thesis PDF — Open Door coaches read fast and don't need a rehearsed pitch.
- Whatever you built, even half-broken. A rough demo beats a polished slide deck.
- One clinical scenario, even without a department name yet — "an ICU nurse" is specific enough to start.

WORTH KNOWING

You don't need to still be enrolled. Skylab Funding and Prototype2Market are explicitly open to recent alumni, not just current students.

Also at Skylab, not health-specific: Ecotrophelia, EuroTeQathon, Open Innovation X, Next Generation [ ] Action, Technology Leaving No One Behind (accessibility — relevant if your thesis touches assistive tech).

Skylab programmes sourced from skylab.dtu.dk. Clinical-collaborator boxes are suggested integration points, not published hospital process.